

1. Administrative & Study Identification

Full Study Title: A Phase 1/1b Open-label, Multi-center Study of BMS-986506 in Participants with Advanced Clear Cell Renal Cell Carcinoma (ccRCC) **Short Title/Acronym:** First-in-Human Study of BMS-986506 in ccRCC **Protocol Number:** CA242-0001 **NCT Number:** NCT07195682 **Sponsor Name:** Bristol-Myers Squibb Co. **Investigational Product:** BMS-986506 (Oral Medication) **Phase of Development:** Phase 1/1b **Medical Monitor:** Bristol-Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To evaluate the safety profile, determine appropriate dosage levels, and find out if BMS-986506 is safe and tolerable when administered as a monotherapy in participants with advanced Clear Cell Renal Cell Carcinoma (ccRCC). **Secondary Objectives:** To understand the pharmacokinetics (how the drug behaves in the body) and observe preliminary efficacy signals of BMS-986506 in advanced ccRCC.

2.2 Background & Rationale To address the urgent need for novel therapeutic approaches for patients with advanced ccRCC who have exhausted standard options. While treatment advances (immunotherapy and targeted therapies) have improved outcomes, many patients develop resistance or experience disease progression. This trial evaluates BMS-986506 as a new oral medication to provide meaningful clinical benefits for these patients.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Single-arm (Dose escalation and expansion) **Blinding:** Open-label **Randomization:** N/A (Sequential dose escalation/expansion cohorts)

3.2 Planned Enrollment & Duration **Target N\$:** Approximately 125 adult participants **Number of Sites:** Multi-center (Global, including various US locations) **Estimated Study Start:** [Study currently Recruiting] **Estimated Primary Completion:** [Date to be confirmed per protocol]

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years with an Eastern Cooperative Oncology Group (ECOG) performance status of 0 to 1. 2. Histologically confirmed diagnosis of locally advanced or metastatic ccRCC. 3. **For Part 1:** Must have received at least two different prior treatment regimens, including both immunotherapy and a targeted therapy. 4. **For Part 2:** Must have undergone at least one standard treatment plan that included both a PD-1/L1 inhibitor and a VEGF-TKI (either in combination or sequentially).

4.2 Exclusion Criteria 1. Inability to administer and/or tolerate oral medication without chewing, breaking, crushing, or otherwise altering the product dosage form. 2. **For Part 2A:** Received more than 3 prior systemic regimens for locally advanced or metastatic ccRCC, including prior treatment with HIF2a inhibitors. 3. Hypoxia defined by a pulse oximeter reading $< 92\%$ at rest, or requiring intermittent/chronic supplemental oxygen. 4. Received colony-stimulating factors (e.g., G-CSF, GM-CSF or recombinant EPO) within 28 days prior to the first dose of study intervention.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: BMS-986506 administered at assigned dose levels during Part 1A (dose escalation) to find the optimal treatment level, followed by target doses in Parts 2A and 2B (dose expansion). **Route of Administration:** Oral. **Duration of Treatment:** Continuous until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing disease-related symptoms. **Prohibited:** Concurrent anti-cancer therapy, colony-stimulating factors within 28 days of the first dose, and concomitant medications known to severely interact with the investigational product's absorption or metabolism.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, ECOG Status, Baseline Tumor Imaging, Oxygen Saturation Check
Baseline	Day 0	Final eligibility confirmation, First Dose of BMS-986506, PK/Safety Labs
Treatment Cycles	Every 2-4 Weeks	Safety Labs, Efficacy Assessment (RECIST v1.1), AE/DLT Monitoring, PK blood draws
End of Study	+30 Days post-last dose	Final Vital Signs, Exit Interview, IP Accountability, Safety Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence of Adverse Events (AEs), Serious Adverse Events (SAEs), and Dose-Limiting Toxicities (DLTs) to determine the Maximum Tolerated Dose (MTD) and Recommended Phase 2 Dose (RP2D). **Measurement Method:** Clinical assessment evaluated by NCI CTCAE criteria.

7.2 Secondary & Exploratory Endpoints Pharmacokinetic parameters (e.g., C_{max} , AUC) of oral BMS-986506. Preliminary Objective Response Rate (ORR) and Progression-Free Survival (PFS) via RECIST v1.1.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring from informed consent through the safety follow-up period. **Serious Adverse Events (SAEs):** Must be reported to the sponsor (Bristol-Myers Squibb) within 24 hours of site awareness. **Data Monitoring Committee (DMC):** An internal Safety Review Committee (SRC) will evaluate DLTs during Part 1A dose escalation to guide dose-level decisions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving ≥ 1 dose of BMS-986506); Efficacy Evaluable Set (patients with measurable disease and ≥ 1 post-baseline scan). **Sample Size Justification:** $N \approx 125$ is based on standard Phase 1/1b dose-escalation (e.g., 3+3 or BLRM design) and subsequent expansion cohort requirements to sufficiently estimate the safety profile and preliminary efficacy. **Handling of Missing Data:** Missing safety data will not be imputed. For tumor response, patients without post-baseline assessments may be considered non-responders.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular on-site and remote monitoring visits to ensure protocol compliance and data integrity. **Audit Trail:** Data entered via an electronic Data Capture (eDC) system with full audit trails, query generation, and eCRF locking mechanisms.

11. Study Identifiers & Governance

Primary ID: CA242-0001 **Registry Number:** NCT07195682 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** First-in-Human Study of BMS-986506 in ccRCC **Official Regulatory Title:** A Phase 1/1b Open-label, Multi-center Study of BMS-986506 in Participants with Advanced Clear Cell Renal Cell Carcinoma (ccRCC)

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced / Metastatic Clear Cell Renal Cell Carcinoma (ccRCC) **Diagnosis Threshold:** Histologically confirmed locally advanced or metastatic ccRCC **Medical Methodology:** Evaluation of novel oral targeted therapy following progression on standard PD-1/L1 and VEGF-TKI regimens **Optimization Target:** Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose (RP2D)

13. Study Procedures & Methodology

Management Pathway: Sequential Dose Escalation (Part 1A) followed by Dose Expansion (Parts 2A/2B) **Education Protocol (HCP):** Investigator meeting and Site Initiation Visits (SIV) focusing on protocol compliance, oral medication adherence, and DLT identification. **Quality Audit Mechanism:** Continuous safety reviews by the sponsor team and primary investigators to regulate dose escalations.

14. Observation & Follow-Up Schedule

Total Duration: Until disease progression or unacceptable toxicity. **Onsite Visit Cadence:** Frequent during Cycle 1 (e.g., weekly) for DLT assessment, transitioning to Day 1 of subsequent treatment cycles. **Radiographic Monitoring Frequency:** Typically every 8 weeks (Q8W) for tumor response evaluation. **Survival Follow-up:** Every 12 weeks post-treatment discontinuation.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				